

Marindi State ONV Immunization

A Comparative Performance Analysis of Campaign Round 1 and Round 2 to Optimize Outbreak Containment and Strengthen Field Execution Strategies.

Vaccine Coverage: State & LGA Analysis

MARINDI STATE NET PROGRESS (ROUND 1 vs ROUND 2)

92.62% 96.28%

3.66% Growth

Structural Performance Shifts

- ✓ **Extensive Regional Gains:** 8 of 12 LGAs expanded coverage. Top gains achieved in **Falpani (+8.78%)**, **Girim (+6.76%)**, and **Belocha (+6.17%)**.
- ! **Targeted Vulnerabilities:** 4 LGAs experienced declines. Most critical drop recorded in **Doriga (-8.56%)**, followed by **Kweli (-2.52%)** and **Lumora (-2.36%)**.



Metric

Calculation:

$$\text{Coverage} = \frac{\text{Children Vaccinated}}{\text{Target Population}}$$

M&E Advisory: Audit the operational framework in Doriga immediately. A baseline shift indicates high micro-level vulnerability despite state-level success.

Geographic Area	R1 Coverage	R2 Coverage	Net Change	Status Profile
Marindi State (Overall)	92.62%	96.28%	+3.66%	Target Achieved
Falpani	87.88%	96.67%	+8.78%	+8.78% (Top Improver)
Girim	89.15%	95.91%	+6.76%	+6.76% (Improved)
Belocha	91.16%	97.34%	+6.17%	+6.17% (Improved)
Ilumba	95.06%	101.08%	+6.02%	+6.02% (Target Met)
Adema	91.72%	97.53%	+5.81%	+5.81% (Improved)
Cunda	90.12%	95.80%	+5.67%	+5.67% (Improved)
Hodu	88.07%	87.68%	-0.39%	-0.39% (Stable Decline)
Lumora	99.66%	97.30%	-2.36%	-2.36% (Decline)
Kweli	90.03%	87.51%	-2.52%	-2.52% (Decline)
Doriga	100.81%	92.26%	-8.56%	-8.56% (Critical Drop)

Non-Compliance & Resolution Rates

Operational Revisit Success

While total non-compliance cases remained relatively stable across rounds, team capacity to resolve refusals improved dramatically, showcasing high field performance:

R1 PENDING PROPORTION

30.27%

247 of 816 children

R2 PENDING PROPORTION

11.73%

✓99 of 844 children

 **Resolution Capacity Surge:** The net pending rate dropped by -18.54 percentage points, reflecting highly optimized inter-round revisit tracking workflows.

 **Declined Resistance Pillars:** Core institutional barriers decreased, most notably **Religious beliefs (-28.4%)** and fear of **Adverse clinical effects (-51.8%)**.

Key Lesson: Systematic, immediate revisits are highly effective. Moving forward, the focus must shift from macro resistance to micro household-level decision-makers.

Non-Compliance Reason Breakdown (R1 vs. R2 Case Counts)



■ Round 1 Cases ■ Round 2 Cases

Recommendations for Next Campaign



1. Micro-Targeted Audits

Establish specialized rapid outbreak response units focusing exclusively on high-risk, declining LGAs (Doriga, Kweli, Lumora, Hodu). Investigate and address local barriers to ensure uniform progress.

- Audit Doriga's target baseline
- Audit cold chain maintenance
- Address local ward variances



2. Household Engagement

Refocus communication strategies from general community advocacy to household-level decision makers. Address the rise in family fatigue and father-led vaccine refusals.

- Initiate father-focused outreach
- Incorporate campaign pauses
- Engage male community leaders



3. Scale Revisit Protocols

Institutionalize the successful Round 2 revisit protocols as a standard campaign process across all wards. Transition the M&E model from passive data collection to active case management.

- Standardize the revisit tracking sheets
- Equip field teams with digital monitors
- Target active pending cases